

LPL - PANDAV NAGAR
SHOP NO-3,A-49,PANDAV NAGAR ; DELHI- 110
PH-011-65257999
DELHI

(Hony) Brig. Dr. Arvind Lal
M.B.B.S., D.C.P.
Padma Shri
FMR HONORARY PHYSICIAN TO THE PRESIDENT OF INDIA



CAP
ACCREDITED



Dr. Vandana Lal
M.D (PATH), IFCAP
Chief of Pathology
SHROMANI AWARD WINNER

Name	: Mr. ANUJ KUMAR	Collected	: 18/3/2018 10:24:00AM
Lab No.	: 137291862	Age: 47 Years	Gender: Male
		Received	: 18/3/2018 10:37:53AM
A/c Status	: P	Reported	: 18/3/2018 6:17:58PM
	Ref By : Dr.SAFDAR JUNG HOSPITAL	Report Status	: Final

Test Name	Results	Units	Bio. Ref. Interval
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Swasth Plus Tax Saver Panel 1

LIVER & KIDNEY PANEL, SERUM

(Spectrophotometry, Indirect ISE)

Bilirubin Total	0.92	mg/dL	0.30 - 1.20
Bilirubin Direct	0.16	mg/dL	<0.20
Bilirubin Indirect	0.76	mg/dL	<1.10
AST (SGOT)	46	U/L	<50
ALT (SGPT)	75	U/L	<50
GGTP	124	U/L	<55
Alkaline Phosphatase (ALP)	104	U/L	30 - 120
Total Protein	7.80	g/dL	6.40 - 8.30
Albumin	4.59	g/dL	3.50 - 5.20
A : G Ratio	1.43		0.90 - 2.00
Urea	26.00	mg/dL	17.00 - 43.00
Creatinine	0.92	mg/dL	0.67 - 1.17
Uric Acid	8.10	mg/dL	3.50 - 7.20
Calcium, Total	9.40	mg/dL	8.80 - 10.60
Phosphorus	3.00	mg/dL	2.40 - 4.40
Sodium	138.00	mEq/L	136.00 - 146.00
Potassium	4.30	mEq/L	3.50 - 5.10
Chloride	103.00	mEq/L	101.00 - 109.00

THYROID PROFILE,TOTAL, SERUM

(CLIA)

T3, Total	1.09	ng/mL	0.60 - 1.81
T4, Total	8.20	ug/dL	5.01 - 12.45
TSH	3.26	uIU/mL	0.35 - 5.50

Note

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
2. Recommended test for T3 and T4 is unbound fraction or free levels as it is metabolically active.
3. Physiological rise in Total T3 / T4 levels is seen in pregnancy and in patients on steroid therapy.



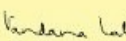
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Clinical Use

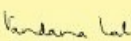
- Primary Hypothyroidism
- Hyperthyroidism
- Hypothalamic - Pituitary hypothyroidism
- Inappropriate TSH secretion
- Nonthyroidal illness
- Autoimmune thyroid disease
- Pregnancy associated thyroid disorders
- Thyroid dysfunction in infancy and early childhood



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URINE EXAMINATION, ROUTINE; URINE, R/E (Automated Strip Test, Microscopy)			
Physical			
Colour	Light Yellow		Pale yellow
Specific Gravity	1.015		1.001 - 1.030
pH	5.5		5.0 - 8.0
Chemical			
Proteins	Nil		Nil
Glucose	Nil		Nil
Ketones	Nil		Nil
Bilirubin	Nil		Nil
Urobilinogen	Normal		Normal
Leucocyte Esterase	Negative		Negative
Nitrite	Negative		Negative
Microscopy			
R.B.C.	Negative		Negative
Pus Cells	Negative		0-5 WBC / hpf
Epithelial Cells	Few		Few
Casts	Nil		Nil /lpf
Crystals	Nil		Nil
Others	Nil		-



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Test Name	Results	Units	Bio. Ref. Interval
HbA1c (GLYCOSYLATED HEMOGLOBIN), BLOOD (HPLC, NGSP certified)	5.9	%	

Interpretation

As per American Diabetes Association (ADA)	
Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years . Goal of therapy: < 7.0 . Action suggested: > 8.0 Age < 19 years . Goal of therapy: <7.5

- Note:** 1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled .
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations.

ADA criteria for correlation between HbA1c & Mean plasma glucose levels

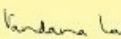
HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126
7	154
8	183



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Test Name	Results	Units	Bio. Ref. Interval
9	212		
10	240		
11	269		
12	298		



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Test Name	Results	Units	Bio. Ref. Interval
HEMOGRAM (Electrical Impedance & VCS, Capillary photometry,Photometry)			
Hemoglobin	13.70	g/dL	13.00 - 17.00
Packed Cell Volume (PCV)	40.70	%	40.00 - 50.00
RBC Count	4.71	mill/mm3	4.50 - 5.50
MCV	86.30	fL	80.00 - 100.00
MCH	29.00	pg	27.00 - 32.00
MCHC	33.60	g/dL	32.00 - 35.00
Red Cell Distribution Width (RDW)	13.10	%	11.50 - 14.50
Total Leukocyte Count (TLC)	7.50	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	61.40	%	40.00 - 80.00
Lymphocytes	29.90	%	20.00 - 40.00
Monocytes	5.70	%	2.00 - 10.00
Eosinophils	2.60	%	1.00 - 6.00
Basophils	0.40	%	<2.00
Absolute Leucocyte Count			
Neutrophils	4.61	thou/mm3	2.00 - 7.00
Lymphocytes	2.24	thou/mm3	1.00 - 3.00
Monocytes	0.43	thou/mm3	0.20 - 1.00
Eosinophils	0.20	thou/mm3	0.02 - 0.50
Basophils	0.03	thou/mm3	0.01 - 0.10
Platelet Count	246.0	thou/mm3	150.00 - 450.00
ESR	21	mm/hr	0 - 15

Note

- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
- Test conducted on EDTA whole blood



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GLUCOSE, FASTING (F), PLASMA (Hexokinase)	108.00	mg/dL	70.00 - 100.00

LIPID SCREEN, SERUM
(Spectrophotometry)

Cholesterol, Total	160.00	mg/dL	<200.00
Triglycerides	222.00	mg/dL	<150.00
HDL Cholesterol	42.70	mg/dL	>40.00
LDL Cholesterol, Calculated	72.90	mg/dL	<100.00
VLDL Cholesterol, Calculated	44.40	mg/dL	<30.00

Interpretation

REMARKS	TOTAL CHOLESTEROL in mg/dL	TRIGLYCERIDE in mg/dL	LDL CHOLESTEROL in mg/dL
Optimal	<200	<150	<100
Above Optimal	-	-	100-129
Borderline High	200-239	150-199	130-159
High	>=240	200-499	160-189
Very High	-	>=500	>=190

Note

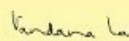
- Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
- ATP III recommends a complete lipoprotein profile as the initial test for evaluating cholesterol.
- Friedewald equation to calculate LDL cholesterol is most accurate when Triglyceride level is <400 mg/dL. Measurement of Direct LDL cholesterol is recommended when Triglyceride level is >400 mg/dL.



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Test Name	Results	Units	Bio. Ref. Interval
IRON STUDIES, SERUM (Spectrophotometry)			
Iron	122.00	µg/dL	65.00 - 175.00
Total Iron Binding Capacity	321.00	µg/dL	250.00 - 425.00
Transferrin Saturation	38.01	%	20.00 - 50.00

Comments

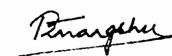
Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.



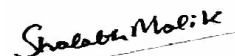
Dr. Anil Arora
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Dr Himangshu Mazumdar
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Consultant Biochemist - NRL



Dr. Nimmi Kansal
MD (Biochemistry)
HOD Biochem & IA - NRL



Dr. Shalabh Malik
MD (Microbiology)
National Head - Microbiology &
Serology - NRL

-----End of report -----

IMPORTANT INSTRUCTIONS

*Test results released pertain to the specimen submitted.*All test results are dependent on the quality of the sample received by the Laboratory.
*Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician.*Sample repeats are accepted on request of Referring Physician within 7 days post reporting.*Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.*Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.*Test results may show interlaboratory variations.*The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s).*Test results are not valid for medico legal purposes. *Contact customer care Tel No. +91-11-39885050 for all queries related to test results.

